Response to reviewers

Point-by-point response table

Foundations of Clinical Research · Session 8 (8.2) and the individual peer review after the course · Template version 1.0

Journal: *[Journal name]*
Manuscript ID: *[e.g. JRNL-2026-0123]*
Title: *[Full title]*
Date: *[DD/MM/YYYY]*
Decision: ☐ Minor revision ☐ Major revision ☐ Reject and resubmit

Tone and method (delete this box)

- Wait a day before replying. Then thank the reviewers once, briefly. They worked for free.
- Answer EVERY comment, in order, numbered (R1.1, R1.2 …). Copy each comment verbatim.
- Say what you changed and where (page and line in the revised manuscript), and quote the new text.
- Agree where you can. If you disagree, do it politely, with evidence or references, and offer a compromise (e.g. a sentence in the limitations). Never argue about the reviewer.
- Show changes in the manuscript with tracked changes or highlighting, as the journal asks.
- Re-check that the abstract, tables and text still match after the changes.

Letter to the editor

Dear Dr *[Editor]*, thank you for the opportunity to revise our manuscript "*[Title]*". We are grateful to the reviewers for their helpful comments. We have addressed every point below and believe the manuscript is clearer as a result. The main changes are: *[1–3 main changes]*. Page and line numbers refer to the revised manuscript with tracked changes.

Worked examples (fictional)

Two agreements and one polite disagreement, for an imaginary cohort paper on fluid balance.

#	Reviewer comment (verbatim)	Our response	Change made (page, line)
R1.1	*The authors state that a positive fluid balance increases mortality. This is an observational study and cannot show causation.*	*Thank you; we agree. We have replaced causal wording throughout with "was associated with", including in the title and abstract, and added confounding by indication to the limitations.*	*Title p1; Abstract p2 l.31; Discussion p14 l.312–318*
R1.2	*How were missing lactate values handled?*	*Lactate was missing for 23 of 412 patients (5.6%). We now report this in Table 1 and the Methods. The main analysis used multiple imputation; a complete-case analysis gave a similar result (OR 1.21 vs 1.18), shown in new Supplementary Table S3.*	*Methods p8 l.170–176; Table 1; Suppl. Table S3*
R2.1	*Please add a subgroup analysis by age group.*	*We appreciate the suggestion. Age subgroups were not pre-specified, and with 68 deaths the analysis would have very low power and a high risk of a chance finding. We therefore respectfully prefer not to add it, and now mention age as a possible effect modifier for future studies.*	*Discussion p15 l.340–343*

Comments from the editor

#	Reviewer comment (verbatim)	Our response	Change made (page, line)
E.1	*[Paste comment]*	*[Response]*	*[p __, l. __]*
E.2	*[Paste comment]*	*[Response]*	*[p __, l. __]*
E.3	*[Paste comment]*	*[Response]*	*[p __, l. __]*

Reviewer 1

#	Reviewer comment (verbatim)	Our response	Change made (page, line)
R1.1	*[Paste comment]*	*[Response]*	*[p __, l. __]*
R1.2	*[Paste comment]*	*[Response]*	*[p __, l. __]*
R1.3	*[Paste comment]*	*[Response]*	*[p __, l. __]*

Reviewer 2

#	Reviewer comment (verbatim)	Our response	Change made (page, line)
R2.1	*[Paste comment]*	*[Response]*	*[p __, l. __]*
R2.2	*[Paste comment]*	*[Response]*	*[p __, l. __]*
R2.3	*[Paste comment]*	*[Response]*	*[p __, l. __]*